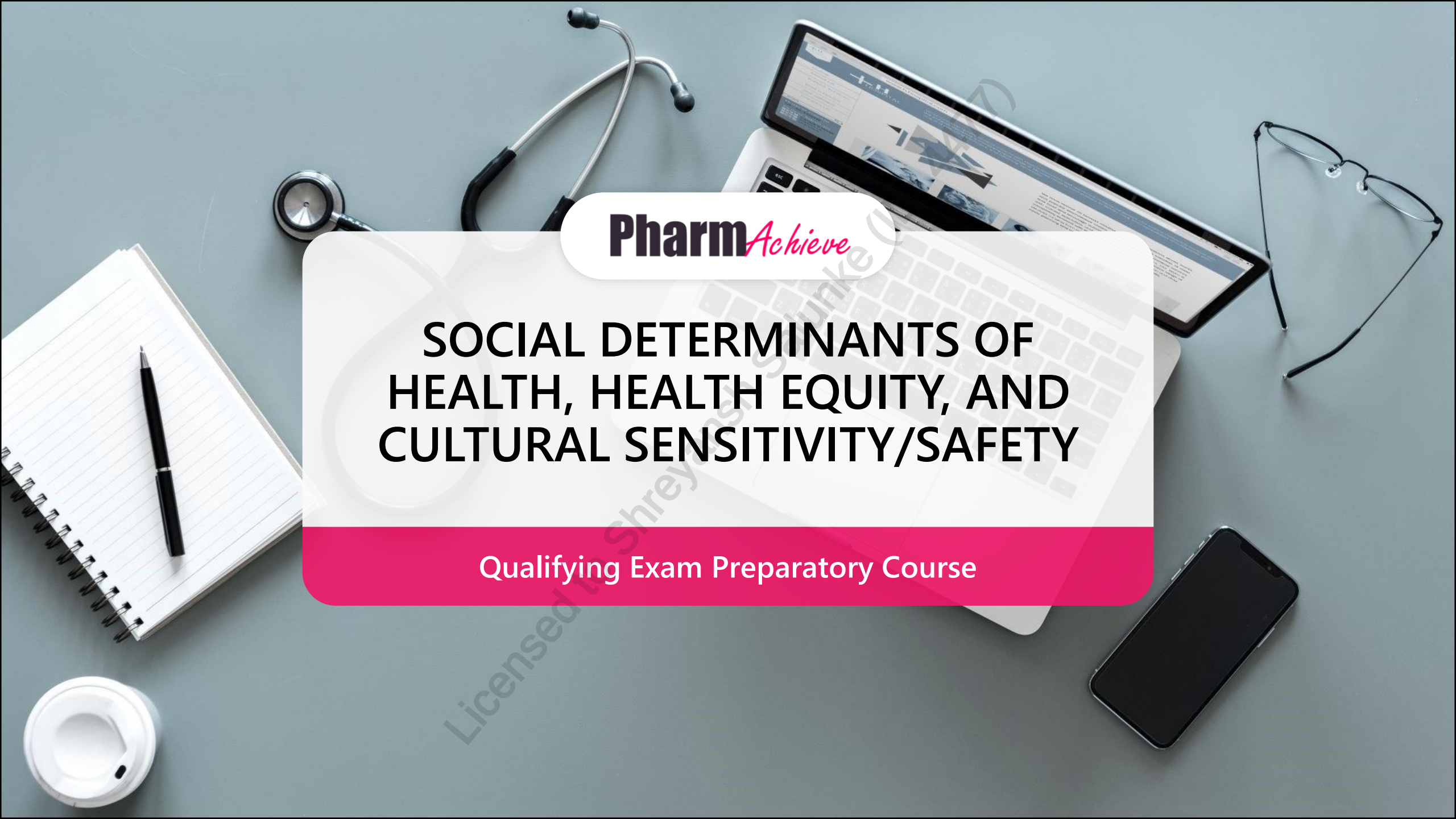




Pharm*Achieve*

SOCIAL DETERMINANTS OF HEALTH, HEALTH EQUITY, AND CULTURAL SENSITIVITY/SAFETY

Qualifying Exam Preparatory Course

COMPETENCIES COVERED IN THIS PRESENTATION...

Providing Care:
Clinical Care

Providing Care:
Distribution

Knowledge
& Expertise

Communication
& Collaboration

Leadership &
Stewardship

Professionalism

LEGEND

2SLGBTQI+	2-Spirit, Lesbian, Gay, Bisexual, Transgender, Queer, Intersex, and Additional People who Identify as part of Sexual and Gender Diverse Communities
ACE	Angiotensin-Converting Enzyme
ARB	Angiotensin II Receptor Blocker
FNHA	First Nations Health Authority
GI	Gastrointestinal
ISC	Indigenous Services Canada (Federal Government organization that provides services to certain Aboriginal peoples of Canada)
NIHB	Non-Insured Health Benefits Program
NSAID	Non-Steroidal Anti-Inflammatory Drug
SDOH	Social Determinants of Health
SGLT2	Sodium-Glucose Cotransporter-2
TCA	Tricyclic Antidepressant
TRC	Truth and Reconciliation Commission

TABLE OF CONTENTS

Differences in health between populations

- Health Inequality Tool

Social Determinants of Health (SDOH)

- Common SDOH
- Example of complex interplay among SDOH

Health Equality vs. Health Equity

- PHAC Marginalized populations

Who are the Indigenous peoples of Canada?

- Historical trauma and its lasting effects
 - Residential Schools
 - Indian Hospitals
 - Birth Alerts
 - 60's Scoop
- Evolving language used to describe Indigenous populations

• Types of Indigenous Healers & Elder Protocol

- Some Indigenous Spiritual Practices that Can Affect Care
- Some cultural practices and counselling points
- Non-Insured Benefits Program

Cultural Safety Continuum

Cultural Cases

LEARNING OBJECTIVES

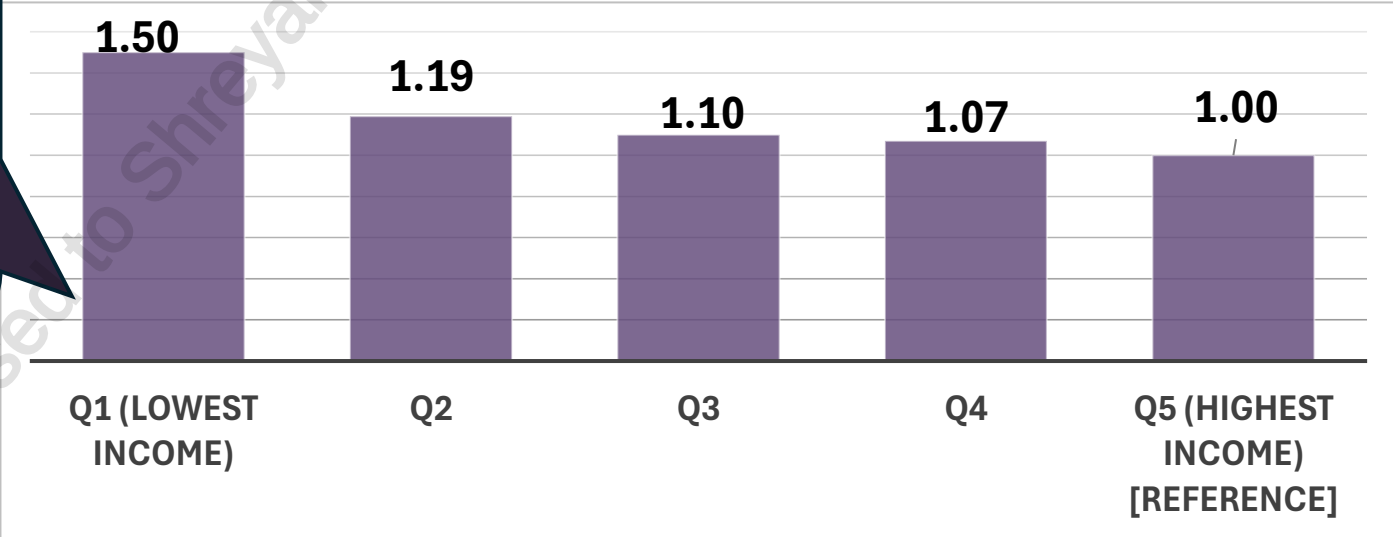
- Define health equity and health inequity
- Understand the impact of social determinants of health
- Gain awareness of Indigenous health needs
- Develop cultural safety and sensitivity skills

SOME CANADIAN EXAMPLES

Health varies considerably between populations.

- (1) **Life expectancy and Indigeneity:** Average Indigenous life expectancy (71.2) is **10.5** years lower than non-Indigenous (81.7)
- (2) **Income and infant mortality:** higher % relative to highest income quintile

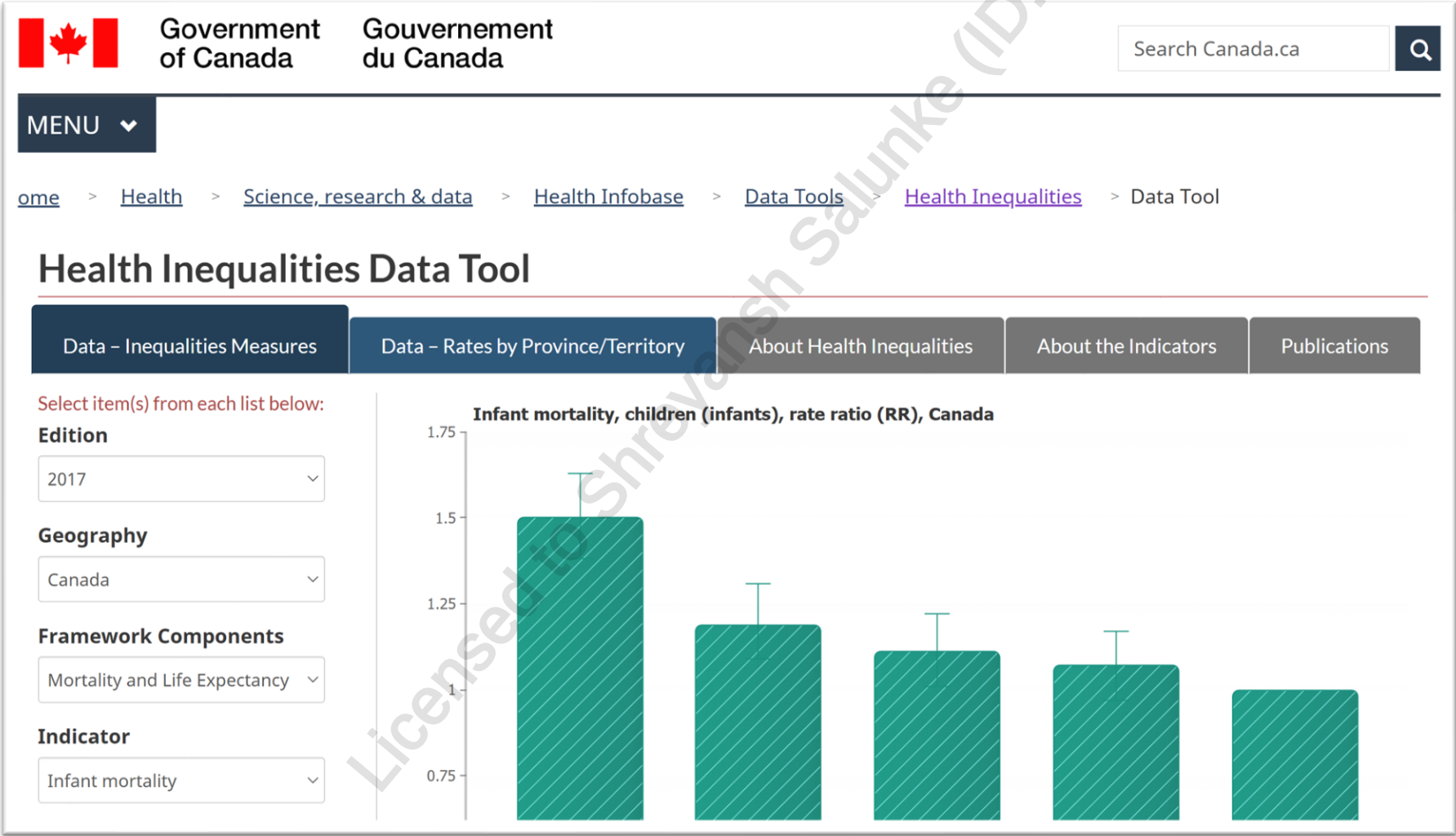
Lowest income Canadians have infant mortality which is 1.5 times that of highest income Canadians.



Graph: © PharmAchieve using Pan-Canadian Health Inequalities Data Tool data
Copyright © 2026 PharmAchieve Corporation Ltd.

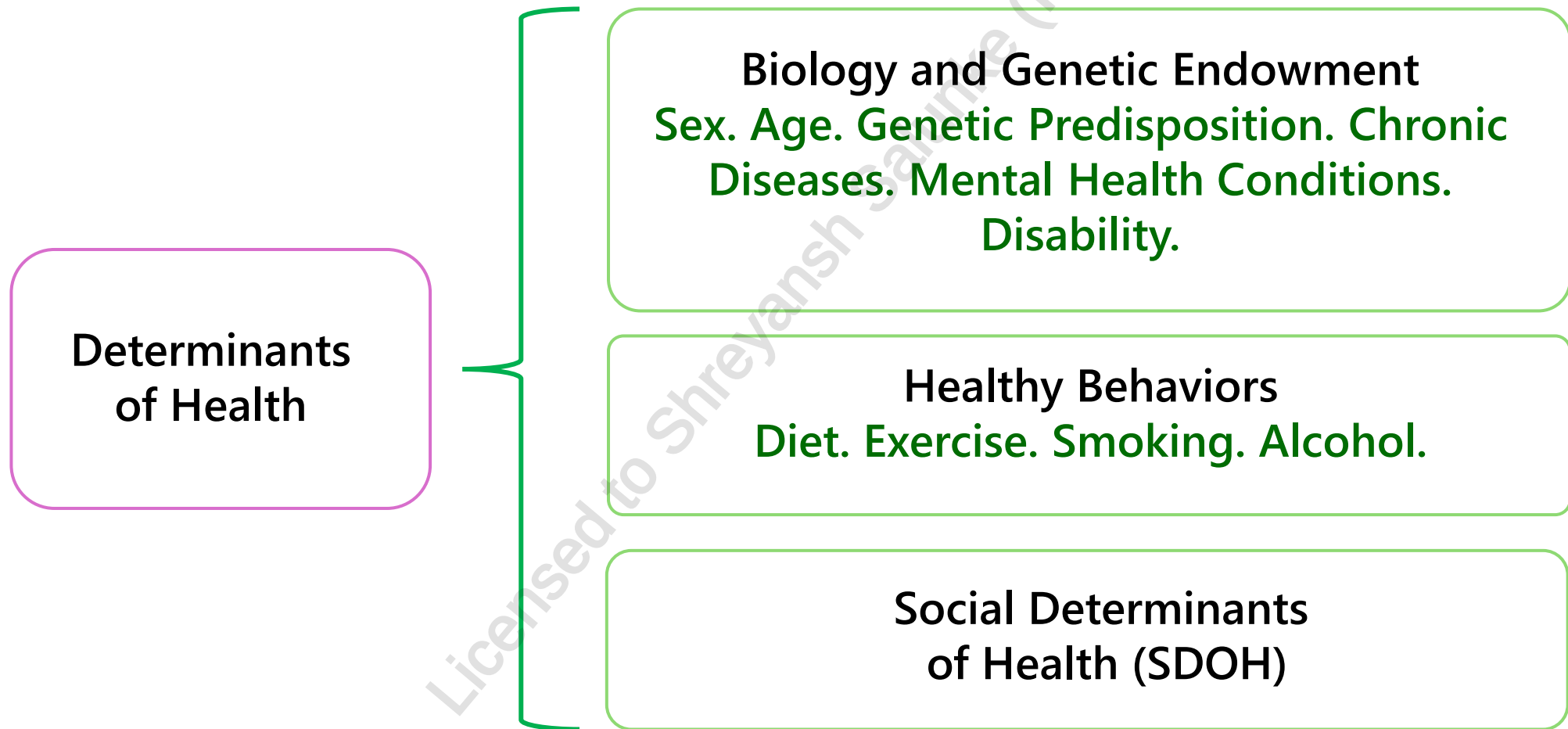
HEALTH INEQUALITY TOOL

https://health-infobase.canada.ca/health-inequalities/data-tool/



DETERMINANTS OF HEALTH

What factors affect the health of individuals and populations?



SOCIAL DETERMINANTS OF HEALTH (SDOH)

All the non-medical factors that affect health apart from sex, genetics, behaviour.

WHO: “conditions in which people are born, grow, live, work, and age”

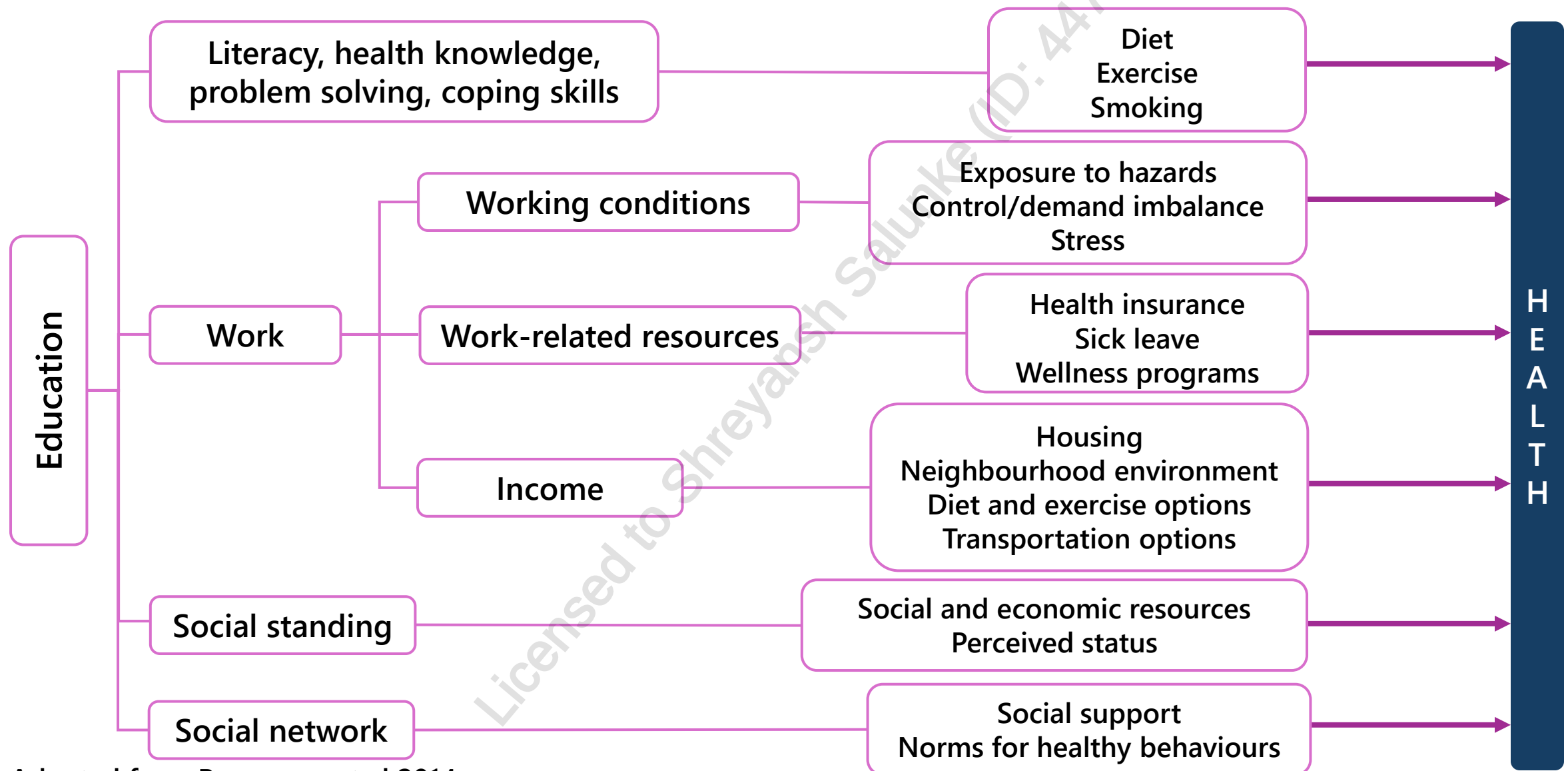
SDOH Factor	How it affects health?
Income	Higher income → better access to resources like quality food, housing, education, and healthcare → longer life and have more years in good health
Employment and Working Conditions	Job security, working conditions (exposure to hazards, stress levels), work-life balance impact health
Education and Literacy	See next slide
Early Childhood Development	Access to early learning and childcare, nurturing family environment, and adverse childhood experiences shape lifelong health and well-being
Physical Environment	The physical surroundings in which people live, work, and play, including a) Housing: Safe, affordable, and adequate housing (including sanitation) b) Environmental Quality: Clean air, water, and soil; absence of toxins c) Access to Green Spaces: Parks, trails, and natural areas promoting physical activity and mental well-being

SOCIAL DETERMINANTS OF HEALTH (SDOH)

All the non-medical factors that affect health apart from sex, genetics, behaviour.
WHO: “conditions in which people are born, grow, live, work, and age”

SDOH Factor	How it affects health?
Social Supports	Access to social assistance programs, employment insurance, and other supports that mitigate the negative impacts of poverty and social disadvantages on health
Access to Health Services	Timely and appropriate availability, accessibility, affordability of healthcare services, including primary care, specialized care, mental health services, and preventative care
Food Insecurity	Lack of consistent access to enough safe, nutritious, and culturally appropriate food for an active and healthy life
Social Exclusion	Discrimination, marginalization, and lack of social/community supports negatively impacts health and well-being
Discrimination, racism and historical trauma	Unique health inequities experienced by certain peoples: e.g. Indigenous, 2SLGBTQI+, Black

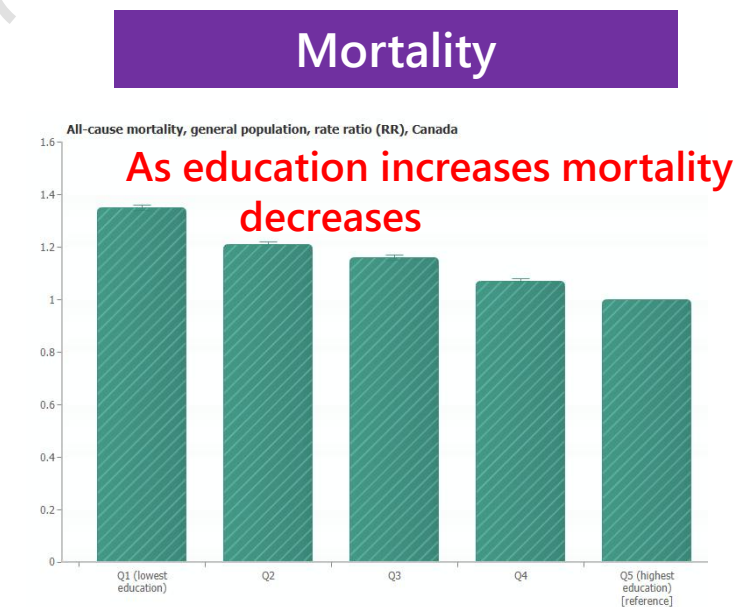
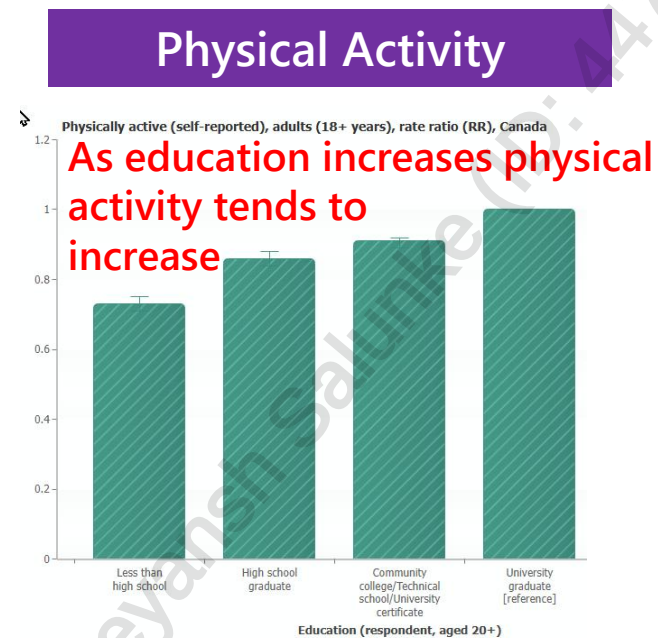
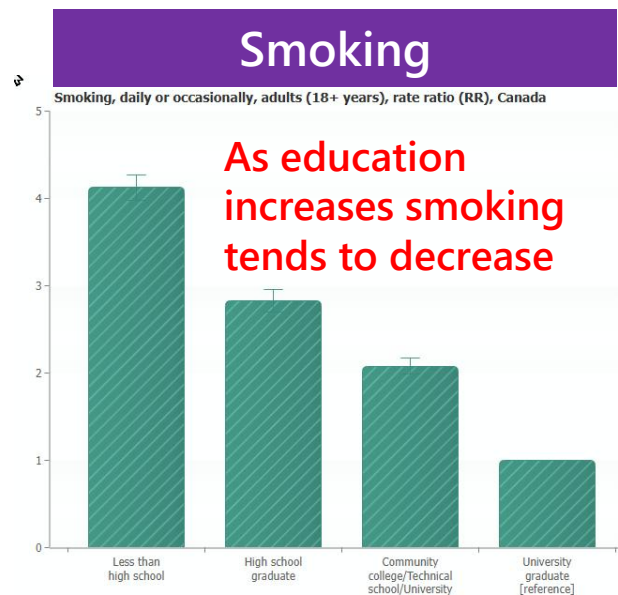
HOW CAN EDUCATION AFFECT HEALTH?



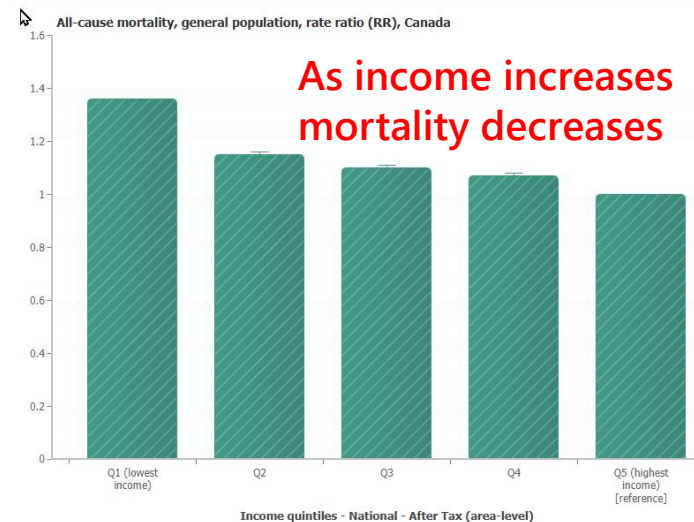
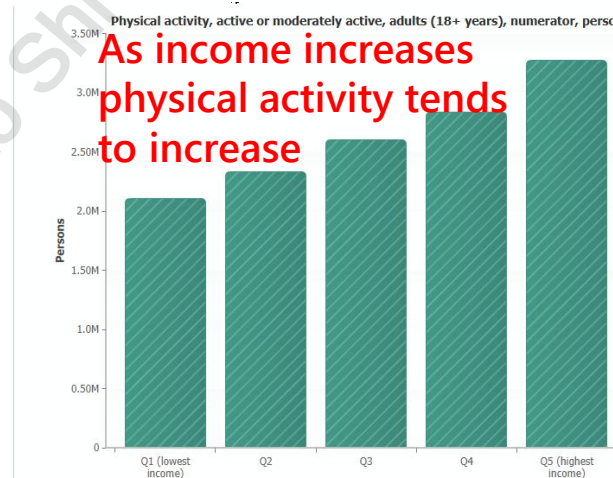
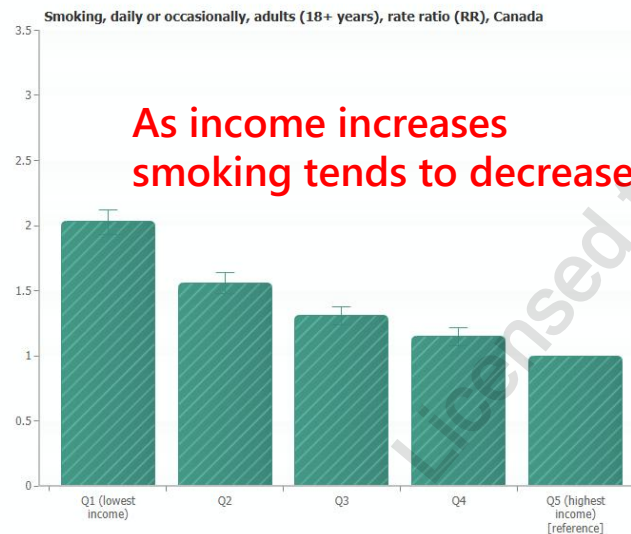
Adapted from Braveman et al 2014.

INCOME AND EDUCATION ARE AMONG THE STRONGEST SDOH

Education



Income



Health Equality

To each an equal share

Health Equity

To each according to need



HEALTH INEQUITIES IN SPECIFIC POPULATIONS

2SLGBTQI+

More likely to experience:

- **Mental health issues** (depression, anxiety, OCD, phobic disorders, suicidality, self-harm, substance use)
- **Violent hate crimes and bullying**
- **Discrimination in the healthcare system and in finding/advancing in employment**
- **Poverty and homelessness**

Indigenous

More likely to experience:

- **Poor access to healthcare**
- **Lack of clean water**
- **Discrimination/mistrust in the healthcare system**
- **Mental health issues**
- **Alcohol/substance Use**
- **Overall: lower quality of care and worse health outcomes**

Black

More likely to experience:

- **Poverty and unemployment**
- **Environmental hazards**
- **Discrimination in the healthcare system**
- **Mistrust in the healthcare system**
- **Poorer health outcomes**

IMPACT OF SDOH ON HEALTH OUTCOMES

1. From a population health perspective in Canada, interventions targeting which area are expected to yield the MOST improvement in reducing health disparities and improving overall health status?
 - a) Expanding the number of hospital beds and emergency departments
 - b) Increasing the availability of publicly funded pharmaceuticals
 - c) Developing new, highly specialized surgical techniques
 - d) Addressing underlying social determinants of health, such as poverty and education levels

2. Which factor is recognized as the primary driver of variations in health status BETWEEN different population groups in Canada?
 - a) Differential access to primary care physicians
 - b) Variations in hospital infrastructure across provinces
 - c) Inequities in social determinants of health
 - d) Differences in genetic predisposition among population groups

IMPACT OF SDOH ON HEALTH OUTCOMES

3. Which of the following factors is generally considered by public health evidence to have the MOST impact on overall population health outcomes in Canada?
- a) The quality and timeliness of acute medical interventions
 - b) Access to advanced diagnostic technologies and specialized healthcare providers
 - c) Social determinants of health (e.g. income level, education, housing stability, food security)
 - d) Individual adherence to prescribed medication regimens

INDIGENOUS HEALTH

Qualifying Exam Preparatory Course



Pharm*Achieve*

WHO ARE THE INDIGENOUS PEOPLES OF CANADA?

Indigenous is an umbrella term used to describe the descendants of the original peoples of Canada before the arrival of European settlers.

The Constitution Act (1982) defined the term "**Aboriginal**" to include 3 groups of peoples with unique history, culture, and languages:

- **Indian (First Nations)** – This includes:
 - **Status Indians** (those registered in the Government's Indian Registry)
 - **Non-Status Indians** (those not registered)
 - The term Indian is a misnomer and is offensive to many Indigenous people, so it has been replaced with the term **First Nations**, which is widely used and accepted
- **Inuit** – the Indigenous of the Arctic regions of Canada (Nunavut, Yukon, Northwest Territories, Labrador, and northern Quebec)
- **Métis** - a distinct people tracing their ancestry to both First Nations peoples (primarily Cree, Ojibwe, and Saulteaux) and European settlers (primarily French, English, Scottish)

WHO ARE THE INDIGENOUS PEOPLES OF CANADA?

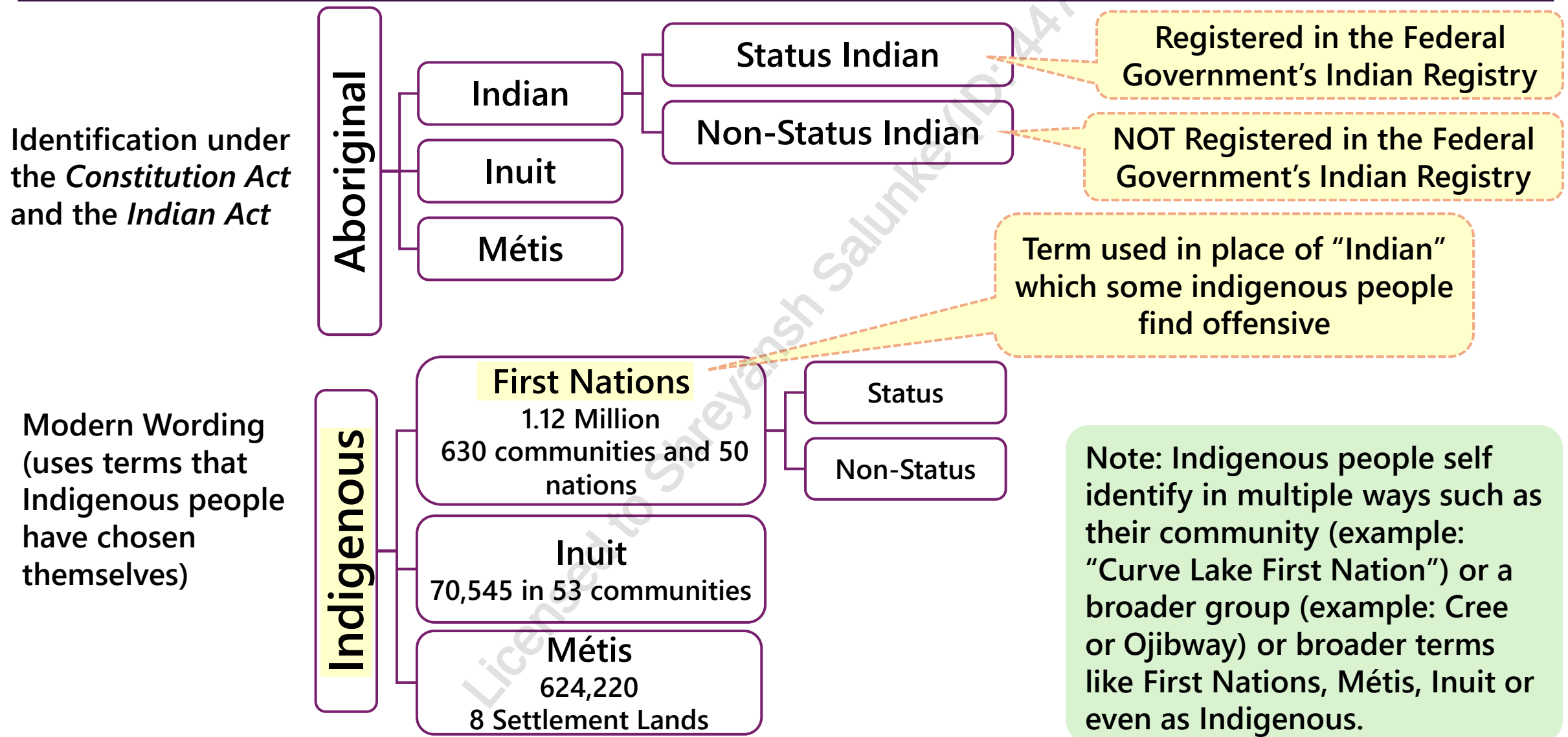
The Constitution Act (1982) recognizes existing “Aboriginal and treaty rights” of “Aboriginal peoples of Canada”

- This recognizes that the Aboriginal rights preceded the existence of Canada
- But it doesn't define what these rights are

What have Aboriginal rights been interpreted to include in the courts?

- Rights to ownership/use of land
- Right to hunt and fish
- Right to practice one's own culture (including traditional Aboriginal medicine)
- Right to establish treaties

EVOLVING LANGUAGE TO DESCRIBE INDIGENOUS POPULATIONS



SOME HISTORICAL CONTEXT

It is not possible to understand Indigenous health, without first understanding historical context of discrimination, colonialism over the last 400 years.

SOME HISTORICAL CONTEXT

Year/Period	Key Historical Event/Policy	Brief Description	Direct and Indirect Impacts on Indigenous Health
17 th -19 th Centuries	Colonial Contact and Expansion	Arrival of European settlers, establishment of trade networks and increased engagement led to the introduction of foreign diseases	• Devastating epidemics (e.g. smallpox, tuberculosis) • Population decline and weakened communities
1876	<i>Indian Act</i>	Law that gave the Canadian Government control over various aspects of Indigenous life, including land, resources, and governance. From 1927 to 1951, the Indian Act banned Indigenous people from hiring lawyers or raising funds to pursue land claims	• Erosion of Indigenous self-determination • Barriers to social and economic development, contributing to health inequities
19 th -20 th Centuries	Establishment of the Reserve System	Indigenous communities were forcibly relocated to much smaller, often less desirable areas called “reserves” limiting their traditional way of life and self-sufficiency	• Increased poverty, food insecurity, and susceptibility to disease due to overcrowding and limited resources

EXAMPLES OF INDIGENOUS COLONIZATION AND DISCRIMINATION

“When the school is on the reserve the child lives with its parents, who are savages; he is surrounded by savages. Indian children should be withdrawn as much as possible from the parental influence.”

- John A. Macdonald, Canada's First Prime Minister (1883)

EXAMPLES OF INDIGENOUS COLONIZATION AND DISCRIMINATION

Residential Schools

- **What:** Government-sponsored, church-run boarding schools that forcibly separated Indigenous children from their families (circa 1846 to 1996)
- **Why:** To force Indigenous children to adopt Euro-Canadian ways while extinguishing their culture
- **How:** Children taught Euro-Canadian culture/language while forbidding them from practicing Indigenous culture/language
- **Impact:** Many children suffered physical, emotional, and sexual abuse. Many died (unmarked graves are now being discovered). Family separation
- **Legacy:** Intergenerational trauma, loss of cultural identity, high rates of mental health issues
- **Truth and Reconciliation Commission (TRC):** Document survivors' experiences and issued 94 Calls to Action for reconciliation



ACCORDING TO THE TRUTH AND RECONCILIATION COMMISSION

ODDS OF DYING FOR CHILDREN
WHILE IN RESIDENTIAL SCHOOLS:

1 in 25*

ODDS OF DYING FOR CANADIAN
SOLDIERS DURING WORLD WAR II:

1 in 26

* Based on the TRC's estimate of 6,000 of the 150,000 students dying while attending residential schools.

EXAMPLES OF INDIGENOUS COLONIZATION AND DISCRIMINATION

Indian Hospitals

- **What:** Racially segregated hospitals, typically under-funded and under-staffed
- **Why:** Initially served as tuberculosis sanatoriums to quarantine Indigenous patients from the non-Indigenous population, but later used as general hospitals. Used as testing sites for experimental vaccines, treatments, and surgeries
- **How:** Patients were often forcibly removed from their communities and hospitalized/quarantined for long periods of time
- **Impact:** Lack of medical autonomy and suppression of Indigenous healing practices. Many patients (particularly children) suffered physical and sexual abuse. Long-term physical impacts from experimentation and treatment. Isolation and removal from community
- **Legacy:** Distrust of healthcare system, loss of culture/language/healing practices, health disparities

EXAMPLES OF INDIGENOUS DISCRIMINATION

What:

- Child welfare warned hospitals about mothers they considered "at risk," often leading to newborns being forcibly taken away at birth

How:

- Alerts issued based on factors like poverty, past trauma, housing instability, history of addiction, without considering systemic causes or protective factors and supports
- Concerns that might be addressed through additional support for non-Indigenous families triggered a birth alert for Indigenous mothers

Legacy:

- Made it hard for Indigenous people to trust the healthcare system or to raise their own children

Truth and Reconciliation: Criticized by TRC. Officially ended on April 14, 2023 when Quebec also banned them

Birth Alerts

Their newborns were taken at birth. Years later, these women still don't know why

3 Ottawa women say 'birth alert' practice left them grappling with trauma decades later



Priscilla Ki Sun Hwang · CBC News ·

Posted: Sep 29, 2022 4:00 AM EDT | Last Updated: September 29, 2022



Click to view CBC
News Article

Article: CBC News

EXAMPLE OF A BIRTH ALERT CASE

"I just had a Cesarian so I could barely walk. **They literally took my son off me while I was breastfeeding my son.** When they came into the room they had 2 security guards at the door. There was a lot of intimidation, panic. I felt trapped, I felt cornered."

EXAMPLES OF INDIGENOUS DISCRIMINATION

60's Scoop

What: systemic removal of thousands of Indigenous children from their families into non-Indigenous care, primarily between the 1950s and 1980s

How:

- Amendments to the *Indian Act* in 1951 gave provinces jurisdiction over child welfare
- Indigenous communities had chronic underfunding making them vulnerable to child welfare interventions
- Social workers often lacking any training on Indigenous culture or history applied based on Euro-Canadian values that disproportionately removed Indigenous children from their parents without informing their band council or community
- Focus was on removing children rather than providing them support
- Birth records not revealed unless both child and parent consented (many children suspecting they were not white were unable to confirm their Indigenous heritage)

Legacy: Loss of trust. Loss of language and culture. Intergenerational trauma

Truth and Reconciliation: TRC calls for awareness, education, support for survivors, and systemic change towards Indigenous self-determination in child welfare



AIM—Adopt Indian-Metis—giving children white parents

By JIM POLING

SASKATOON — (CP) — Louise is a dark-eyed, bright 34-year-old who, although her mind can't comprehend it yet,

is engaged in a desperate battle which will shape her future.

The fight is for security, something she hasn't found in three different homes and

something which her fourth — and probably permanent — parents hope to give her. Louise's fight is different from that of most adopted children because her new

family is white and she is Metis — part Indian and part white.

She represents a challenge not only to her new parents, but to a branch of the Saskatchewan department of welfare called AIM — Adopt Indian-Metis centre.

AIM was established three years ago as a pilot project in the Regina area when a serious backlog of Indian and Metis children under provincial care developed.

During 1966-67, the fiscal year before the project started, only 50 Indian and Metis children were adopted in Saskatchewan.

From April 1, 1969, to Dec.

31, 1969, a total of 140 were placed in permanent homes. Sixty of these were placed by AIM's Regina office and its Saskatoon branch and the rest by the welfare department which handles Indian and Metis adoptions outside the two districts.

One of the questions Louise's prospective parents had to answer before going to AIM was: Aren't there enough problems in adoption without taking a child of another race?

"To most people who come here, race makes no difference," said Allison Vickers, AIM supervisor for the Saskatoon office. "But they are

aware that it does to some people."

Louise's new parents, who have two boys, aged 9 and 10, and a girl 6, were drawn to AIM by its publicity campaign and a long-standing interest in the Indian people.

'PROUD AS PUNCH'

"I'm adopted myself and have wanted children both ways," says Louise's new mother, who wished to remain anonymous to protect her new child. "We felt that if we wanted another child, why produce one when there are so many already available?"

She and her husband wanted another girl and after months

of thought went to AIM because they felt they could help the problem of Indian and Metis children by adopting one.

"At first I thought that when I took her shopping with me I would be apprehensive. But I'm as proud as punch taking her and I expect everyone to like her."

"We haven't met any discrimination yet . . . but perhaps it's discrimination of a form when people say 'Aren't you the good Samaritan?'"

Louise was abandoned at two years of age and lived in two foster homes. When taken into the care of the province she spoke only Cree.

the family and the child can adjust. At the end of that time the child may be returned — though not many are — or the legal rights to the child are transferred to the new parents.

Before AIM was established the number of Indian and Metis children awaiting adoption had been increasing at about 100 a year.

In October, 1969, there were 285 Indian and Metis children under provincial care and by last month 186.

Mrs. Vickers says that AIM is at least keeping ahead of the increase and that the program has boosted the number of adoptions of all types in Saskatchewan.

But now it is to for older children in family groups.

LIKE BIRTH

er children it is birth. It's like a n Oriental marriage contract.

ut love," she has by her brothers who were pre-arrival. She also accepted by the kids.

we can see her e than theirs, but e reason, or care, another race.

arents are con- ren the love and s their natural received, Louise a stable adit,

race and proud e family which e happiness and couldn't have series of foster

A Feature of Ogilvy's Goods and Chattels Sale



Informal See-Through Dining Set

A CHILD IS WAITING

Happy, Playful Girl



Sherri, 3 . . . loves to be hugged and cuddled.

Three-year-old Sherri can quickly win your attention with her wide smile and big brown eyes. An attractive girl, she has straight black hair cut

she is talking, repeating what others say and is more attentive. She is attending a speech therapy program once a week and her foster

A CHILD IS WAITING



JASON, A LOVABLE LITTLE CHARMER . . . is all-boy.

Youngster loves outdoors

Don't let his serious look fool you, twenty-month-old Jason is a happy boy who enjoys playing with his children and adults.

has fine dark brown hair, big brown eyes, chubby cheeks and a dark olive complexion.

For the first months of his

60's Scoop



OUT OF NOWHERE, INTO HERE...

Once this child's life and future was uncertain, lonely. He faced the problems of a Metis child growing up without parents to guide him. Now he lives in a home full of love, free from prejudice. If yours is a childless home, or if you want to complete your family, there is a little Metis boy or girl longing for you. You can become part of AIM and give your lives new meaning.

Prospective adopting parents can rest assured that children offered for adoption are carefully evaluated, to ensure, as far as is humanly possible, that they are in every way normal.



I have often thought what a melancholy world this would be without children.

ADOPT INDIAN METIS Centre AIM
8440 ALBERT BLVD. REGINA, SASKATCHEWAN. TELEPHONE 824-0001
DEPARTMENT OF WELFARE
A SPECIAL PROVISIONAL REGULATION, NATIVE CHILDREN ACT, SASKATCHEWAN

AIM was a government of Saskatchewan program that stands for "Adopt Indian and Métis. Here are some newspaper advertisements from the Montreal Gazette, the *Regina Leader-Post* and *Saskatoon Star Phoenix*.

TRUTH AND RECONCILIATION COMMISSION

What: TRC was established as part of a legal settlement to uncover the truth about Canada's residential school system. It was funded by the settlement. 2 of 3 leaders were of Indigenous ancestry

How:

- Gathered testimony from over 6,500 residential school survivors and others affected
- Held National Events across Canada to raise awareness and facilitate sharing
- Conducted extensive research into historical records and government policies
- Documented the history, purpose, operation, and consequences of the residential school system

Key recommendations:

- Released a comprehensive final report in 2015
- Issued 94 Calls to Action to address the legacy of residential schools and advance reconciliation
- Calls to Action cover areas like child welfare, education, language & culture, health, and justice
- Calls 18 to 24 are related to health

TRUTH AND RECONCILIATION COMMISSION

Examples:

- **Action 18:** "We call upon the federal, provincial, territorial, and Aboriginal governments to acknowledge that the current state of Aboriginal health in Canada is a direct result of previous Canadian government policies, including residential schools, and to recognize and implement the health-care rights of Aboriginal people as identified in international law, constitutional law, and under the Treaties."
- **Action 19:** "We call upon the federal government, in consultation with Aboriginal peoples, to establish measurable goals to identify and close the gaps in health outcomes between Aboriginal and non-Aboriginal communities, and to publish annual progress reports and assess long-term trends."
- **Action 22:** "... recognize the value of Aboriginal healing practices and use them in the treatment of Aboriginal patients in collaboration with Aboriginal healers and Elders where requested by Aboriginal patients."
- **Action 23:** We call upon all levels of government to:
 - i. Increase the number of Aboriginal professionals working in the health-care field
 - ii. Ensure the retention of Aboriginal health-care providers in Aboriginal communities
 - iii. Provide cultural competency training for all healthcare professionals.

SOME COMMON TERMS FOR INDIGENOUS HEALERS

- Elders:
 - Respected individuals recognized by their community/Nation for wisdom, cultural knowledge, spiritual guidance, and life experience
 - Role and recognition process vary significantly between Nations
Note: Not all Indigenous seniors are Elders; it is an earned and recognized role
- Knowledge Keepers:
 - Individuals (of any age) recognized for holding specific traditional knowledge (e.g. language, history, ceremonies, traditional medicine, law, specific skills)
 - May or may not also be an Elder
- Healers/Medicine Persons:
 - Individuals recognized for specialized knowledge of traditional medicines, healing practices, and ceremonies

Terminology varies. Protocols for engagement are often highly specific and must be learned locally, often requiring introduction or facilitation by community members

NON-INSURED HEALTH BENEFITS PROGRAM (NIHB)

Provides [eligible First Nations and Inuit clients](#) with coverage for a range of health benefits not covered through other:

- Social programs
- Private insurance plans
- Provincial or territorial health insurance

The program provides coverage for the following medically necessary goods and services:

- [Vision care](#)
- [Dental care](#)
- [Mental health counselling](#)
- [Medical supplies and equipment](#)
- [Prescription drugs and select over-the-counter products](#)
- [Medical transportation](#) to access medically required health services not available on reserve or in the community of residence



NIHB supplements provincial/territorial health coverage and covers some services that other Canadians must pay for privately BUT:

- NIHB is viewed by many First Nations/Inuit as fulfilling treaty promises
- Many Indigenous people do not receive these benefits owing to poor access, jurisdictional disputes, and culturally unsafe care resulting in them avoiding the healthcare system

Source: Dr. Jaris Swidrovich

IDENTIFICATION NEEDED TO ACCESS NIHB PROGRAM

Indian Status Card



Inuit

- Inuit Land Claim Beneficiary Number or
- Letter of recognition from the organization

***Note:** Métis patients do not have access to NIHB benefits.

***Note:** Not all First Nations/Inuit are covered under the NIHB program. Many do not have coverage due to:

- Loss of status through enfranchisement
- Not meeting the provisions of the *Indian Act* or being unable to prove eligibility
- Personal choice
- Etc.

NON-INSURED HEALTH BENEFITS DRUG BENEFIT LIST



 EXPRESS SCRIPTS®

[Provider](#) ▾ [Client](#) [About Us](#) ▾ [Alerts](#) [Reports](#) [Contact Us](#)

[Log In](#)

[Home](#) | [Pharmacy](#) | [Drug Benefit List](#)

Drug Benefit List

[Online Drug Benefit List](#) | [Download Drug Benefit List](#)

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Online Drug Benefit List

DIN/NPN/PDIN	Item Name	Strength	Dosage Form ▾
Chemical Name	AHFS Class	Manufacturer	
Coverage Benefit Status ▾			

CONTINUUM OF CULTURAL SAFETY

	Cultural Awareness	Cultural Sensitivity	Cultural Competence	Cultural Safety
				
What signifies this stage?	Awareness that cultural differences exist	Exhibiting skills and behaviours considered polite/respectful by other culture	Developing skills and behaviours for delivering effective care to diverse populations; self-reflection	"Environment free of racism, discrimination and bias where patient <i>feels</i> safe, and respected." -Source: FNHA
Focus	Differences (self vs. other)	Respecting differences	Learning about other culture; self reflection on own values and how they impact care	Patient experience; power dynamics; addressing power imbalances and systemic barriers (racism, discrimination)
Goal	Acknowledge diversity	Avoid offense; Build rapport	Providing effective care to diverse populations	Creating a safe and equitable environment (defined <i>by patient</i>); Building trust
Caution	May reinforce "othering"	Focus on politeness may miss deeper needs/inequities	Risk of "checklist" approach or stereotyping; implies mastery is possible	Requires ongoing commitment & system change

DO'S AND DON'TS

DON'T 	DO 
Assume that traditional medicine is ineffective or inferior to Western medicine	• Remember: many Western medicines were first discovered or used by Indigenous peoples • Understand that a lack of research studies/data does not equal lack of efficacy
Assume that the patient must choose between Western or traditional medicine, or discourage the use of traditional medicine if desired by the patient	• Understand patient may use both; and where desired by the patient engage in shared decision-making about how they can potentially work together safely • Respect the patient's beliefs and autonomy
Ignore traditional medicines/practices during counseling or inquire using vague questions	• Explain the rationale for asking and how information will be used/documented • Respectfully ask about all things the patient uses for health by giving examples (e.g. sweat lodges, fasting, smudging, plant medicines)
Assume that all Indigenous people use the same medicines in the same way. (i.e. Pan-Indigenization)	• Recognize that the use of traditional medicines varies by community, geographic location, history, and cultural traditions; there is no "one size fits all"
Reach out to a patient's traditional healer or Elder without explicit permission and facilitation	• With explicit patient consent and guidance, explore respectful collaboration with a healer/Elder if desired by the patient

DO'S AND DON'TS

DON'T 	DO 
Assume all information about traditional medicines can be found in databases or have standardized doses/ingredients	• Recognize that information may primarily come from the patient/local knowledge; acknowledge the limitations of standard references • Ask the patient how much they take, how they administer it, and how often
Rely solely on databases for interaction checking	• Gather information directly from the patient (if they are comfortable sharing) to learn about the traditional medicine • If unable to find in databases, assess theoretical risks based on anticipated pharmacologic effects and/or closest matching Western drug properties (e.g. adverse effects, MOA, metabolism)
If concerned about potential interactions or side effects, ignore/disregard the traditional medicine or simply advise patient not to take it	• If interaction risks with the Western drug are significant and/or unknown, discuss strategies like enhanced monitoring • If the traditional medicine cannot be used safely, discuss your specific concerns with the patient • When in doubt, "start low and go slow" with Western or traditional medicines

SOME CULTURAL PRACTICES AND COUNSELLING POINTS

Cultural Practice	Drug Classes and Associated Concerns	Key Counselling Considerations
Fasting	• Hypoglycemia (Low Blood Sugar): Medications with hypoglycemic effects, particularly – insulin, sulfonylureas (e.g. gliclazide, glyburide), meglitinides (e.g. repaglinide) • Altered Drug Absorption/Timing: drugs requiring food for optimal absorption (e.g. itraconazole caps, ziprasidone, rilpivirine) or to minimize GI effects (e.g. NSAIDs) • Dehydration (if fluid intake also restricted): diuretics, ACE Inhibitors/ARBs, SGLT2 inhibitors (See Sweat Lodge: Dehydration)	• Discuss dose adjustments prior to fasting • Counsel on hypoglycemia recognition/management • May require more frequent blood glucose/blood pressure monitoring and/or temporarily holding medication • Review medication list for food requirements. For short-term medications, consider alternatives • Advise on appropriate timing around fasting periods if medication cannot be held • Clarify if fast includes fluids. Advise hydration if permitted

SOME CULTURAL PRACTICES AND COUNSELLING POINTS

Cultural Practice	Drug Classes and Associated Concerns	Key Counselling Considerations
Sweat Lodge Ceremony	• Dehydration, Electrolyte Imbalance, Heat Stress: diuretics (e.g. furosemide, hydrochlorothiazide), ACE inhibitors/ARBs (e.g. ramipril, losartan), SGLT2 Inhibitors (e.g. empagliflozin), laxatives • Altered Thermoregulation: anticholinergics (e.g. oxybutynin, TCAs), antipsychotics (e.g. olanzapine, risperidone), beta-blockers (can blunt heat response) • Blood Pressure Fluctuations (Orthostatic Hypotension): antihypertensives (all classes), alpha-blockers (e.g. tamsulosin, prazosin), nitrates	• Advise on adequate hydration before/during/after • Discuss potential need to hold doses (esp. diuretics) on the day of the ceremony, if clinically appropriate and safe (requires assessment) • Monitor for dizziness, hypotension. Consider blood pressure monitoring if feasible • Counsel on recognizing signs of heat exhaustion/stroke • Caution regarding reduced ability to sweat or perceive heat • Advise slow positional changes after ceremony

CULTURAL SAFETY CASES

1. Which of the following is an example of providing culturally safe care?
 - a) Using an interpreter to discuss treatment options with a patient who doesn't speak English
 - b) Displaying welcoming signage, symbols, and artwork in your office
 - c) Engaging in self-reflection and unlearning your own biases and assumptions
 - d) All of the above are examples of practicing culturally safe care

2. A patient shares that they will be participating in a fast for cultural reasons. The fast will last for 24 hours, and they are permitted to drink water. They are taking metformin 1000mg po twice daily for type 2 diabetes (recent A1C 6.6%). What is the most appropriate recommendation?
 - a) Tell the patient to stop metformin 2 days before the fast, and resume taking it after
 - b) Review the importance of hydration, monitoring blood sugar, and situations that would require breaking the fast
 - c) Advise the patient that it is unsafe to fast
 - d) Tell the patient that they are exempt from the fast because they have diabetes

CULTURAL CASES

Remember: These guidelines are things to look out for, but each person is an individual with their own preferences.



Culture	Muslim	Hindu	Sikh	Jewish	Indigenous
Example	Fasting during Ramadan with diabetes	Vegetarian – May require non-animal sourced gelatin	Hair is sacred and never cut (Kesh)	Dietary requirements	Smudging in an inpatient care facility

CULTURAL CASES

Remember: These guidelines are things to look out for, but each person is an individual with their own preferences.



Culture	Muslim	Hindu	Sikh	Jewish	Indigenous
Example	Fasting during Ramadan with diabetes	Vegetarian – May require non-animal sourced gelatin	Hair is sacred and never cut (Kesh)	Dietary requirements	Smudging in an inpatient care facility
References / Supports	Diabetes Canada guidelines for fasting	Drug Product Database (lists ingredients)	Seek alternatives and if still necessary provide advance notice and seek consent	Drug Product Database (lists ingredients)	Advance notice, signage, temporarily bypass the fire alarm system, use ventilation or hold outdoors

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CHANGE LOG

January 2026

- New lecture

March 2026

- Added slide 42 (cultural safety questions)
- Minor edits to slide 44